Capgras Syndrome

Capgras Syndrome is a rare delusional misidentification syndrome characterized by the belief that an important and familiar person in the individual's life -most commonly a spouse, family member, or close relative- is not actually the real person and has instead been replaced by an impostor who looks exactly like them. Although the patient may acknowledge that the person in front of them looks physically identical, they do not believe that this person is their actual relative. For this reason, Capgras Syndrome is also commonly referred to as the "delusion of doubles." The syndrome was first described in 1923 by French psychiatrists Joseph Capgras and Jean Reboul-Lachaux and was initially termed *l'illusion des sosies* ("the illusion of doubles").

Symptoms and the Patient's Experience

The main symptom of Capgras Syndrome is a delusional belief that a familiar person has been replaced by someone else. The patient may recognize the face and appearance of their spouse or another close relative; however, they may lose the subjective and emotional sense of familiarity associated with that person being the real individual. For example, they may hold a firm belief such as, "This person looks very much like my spouse, but they are not my spouse; they are an impostor who has replaced them." This belief is not merely an ordinary suspicion or uncertainty; the patient is generally strongly convinced of it.

Misidentification may cause anxiety, uneasiness, fear, suspicion, agitation, and anger. Because the patient may believe that their actual relative is deceiving them or that someone has replaced them, they may have difficulty accepting explanations provided by people around them. As the delusion progresses, the patient may engage in behaviors aimed at proving who the "real person" is and may ask the person in front of them to prove that they are actually their spouse or relative. This can lead to deterioration in interpersonal relationships and serious conflicts within the family.

When the patient perceives the misidentified person as a threat, aggressive or violent behaviors may occur. However, it should be noted that not every patient with Capgras Syndrome exhibits violent behavior and that non-violent cases also constitute a significant proportion. In some cases, misidentification may occur not only with people but also with animals or inanimate objects.

The severity and course of symptoms may vary depending on the underlying disorder. A large cohort study published in 2026 reported that Capgras symptoms may worsen particularly during the evening or nighttime and that this may be associated with negative emotional states.

Why Does It Occur?

Capgras Syndrome does not have a single cause. Current evidence suggests that the syndrome is associated particularly with disturbances in processes such as face recognition, emotional response, memory, self-monitoring, and reality evaluation.

One prominent explanation is that although the individual can visually recognize a familiar face, the normal sense of emotional familiarity associated with that person fails to arise. Normally, when we see the face of someone we know, we do not merely identify who they are; we also experience a sense of familiarity and emotional closeness toward that person. In Capgras Syndrome, the connection between face recognition and this emotional response is thought to be disrupted. Thus, the patient may recognize the appearance of the familiar person but fail to experience the expected sense of familiarity and may interpret this discrepancy as meaning that "the real person has been replaced by an impostor."

The right frontal regions, temporal regions, and limbic system are thought to play important roles in this process. More recent neuroimaging findings indicate that Capgras Syndrome may involve widespread bilateral cortical involvement, particularly in association with right frontal dysfunction. Therefore, the syndrome is not regarded merely as a problem of "being unable to recognize a face," but rather as a more complex neuropsychiatric condition involving the combined disruption of processes related to face recognition, emotions, memory, reality evaluation, and the monitoring of one's own thoughts.

Which Disorders Can It Occur With?

Capgras Syndrome may occur on its own or in association with various psychiatric and neurological disorders. Sources have reported associations particularly with Lewy body dementia, Alzheimer's disease, schizophrenia, schizoaffective disorder, bipolar disorder, epilepsy, Parkinson's disease, cerebrovascular diseases, and various forms of brain injury. In a large cohort study published in 2026, 69% of the 204 patients examined had a neurodegenerative disease, with Lewy body dementia (58%) being the most common diagnosis. Psychotic disorders accounted for 9% of the cases. These findings suggest that Capgras Syndrome may be an important clinical manifestation, particularly in the context of neurodegenerative diseases.

Diagnosis and Treatment

The diagnosis of Capgras Syndrome is primarily based on clinical assessment. However, when necessary, laboratory investigations and brain imaging may be performed to identify neurological or medical conditions that may underlie the syndrome. In particular, it is important to investigate dementia, brain lesions, neurodegenerative diseases, and other organic causes.

There is no single standard treatment for Capgras Syndrome. The main reason is that the syndrome may arise as a consequence of different disorders and that treatment studies are limited due to its rarity. The general approach involves treating the underlying condition, using antipsychotic medications when appropriate, and providing psychotherapeutic/psychosocial support.

Rather than directly confronting the patient's belief or attempting to forcibly convince them that it is incorrect, an empathetic and calm form of communication aimed at understanding the fear and anxiety experienced by the patient is recommended. It is particularly important for family members and caregivers to understand the patient's experience. Because direct confrontation with the person whom the patient perceives as an "impostor" may increase anxiety and agitation in some cases, communication should be clear, calm, and reassuring. In patients who pose a risk of harm to themselves or others, more intensive psychiatric assessment and, when necessary, hospitalization may be considered.

In Brief

Capgras Syndrome is a rare delusional misidentification syndrome in which an individual believes that a person they know and love is not the real person and has instead been replaced by an impostor who looks identical to them. The fundamental problem is not simply an inability to recognize a face; rather, it involves a disruption in the connections between face recognition, emotional familiarity, memory, and reality evaluation. Therefore, the patient may recognize the appearance of their relative but may not experience the sense that "this is my relative."

The syndrome may be associated with neurodegenerative diseases such as Lewy body dementia and Alzheimer's disease, as well as schizophrenia and other psychiatric or neurological disorders. Patients may experience anxiety, suspicion, and agitation, and in some cases there may also be a risk of aggression toward the misidentified person. Therefore, early recognition of Capgras Syndrome, investigation of the underlying cause, and appropriate management are important.

References

1) Shah KP, Jain SB, Wadhwa R. Capgras Syndrome. 2023 May 29. In: StatPearls [Internet]. Treasure Island (FL): StatPearls Publishing; 2026 Jan–. PMID: 34033319.

2) Brigo F. Capgras Syndrome in Popular Media: A Cinematic Exploration in Kinds of Kindness (2024). Clin Neuropsychiatry. 2025 Apr;22(2):165-166. doi:

10.36131/cnfioritieditore20250207. PMID: 40401142; PMCID: PMC12090369.

3) Watanabe H, Whitwell JL, Bhaskaran JG, Lowe VJ, Josephs KA. Imposter in the brain: aetiological, clinical and neuroimaging characteristics of Capgras syndrome. Brain. 2026 Apr 7;149(4):1224-1238. doi: 10.1093/brain/awaf378. PMID: 41059767; PMCID: PMC13058470.

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